Referral packet F25: readable copy

Source: F25.pdf (fictional), 12 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

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3	CalAIM Community Supports authorization request	calaim_auth	good	
4	Patient face sheet	face_sheet	good	
5	History and Physical - page 1	h_and_p	good	
6	History and Physical - page 2	h_and_p	good	
7	Infectious disease consult	other	good	
8	Hospitalist progress note	progress_note	good	
9	Nursing note	nursing_note	good	
10	Case management note	social_work	good	
11	Vitals, labs and microbiology	labs	good	
12	Discharge medication list (draft)	med_list	good	



Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Harbor Crest Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Keith Yamamoto, LCSW
Phone: 555-0141
Date: 10/05/2026 11:20
Pages (including cover): 12
Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

54M MSSA bacteremia from L foot cellulitis, on IV cefazolin via PICC x 4 more weeks.
Home infusion arranged per referral form. Hoping for d/c 10/06/2026.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

FAX F25 FICTIONAL DEMO RECORD · NOT A REAL PATIENT PAGE 02/12

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Harbor Crest Medical Center (fictional) Dept/Floor: 6 East Medicine
SW/CM/RN: Keith Yamamoto, LCSW Phone/Pager #: 555-0141
Email: kyamamoto@harborcrest.example Nursing Station #: 555-0142
Authorized by: Dr. P. Adeyemi Phone #:

Patient Information

Patient Name: Luis Doe MRN: HC-5027719 Date of Birth (dd/mm/yyy): 06/30/1972
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Hispanic or Latino
Last 4 # of Social Security: xxx-xx- 0000 Insurance: [redacted] Preferred Language: Spanish / English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
MSSA bacteremia, L foot cellulitis
Admit Date: 09/30/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☒ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 09/30/2026 Results: CXR clear

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ✕ No	Active Infection	✕ Yes* No
*Explain		*Explain MSSA bacteremia - IV cefazolin via PICC	
Self Administer All Meds	✕ Yes No*	Ambulatory	✕ Yes No
Continent of Bowel & Bladder	✕ Yes No	Assistive Device	Yes* ✕ No
Colostomy/Ileostomy	Yes ✕ No	*Explain	
Foley Catheter	Yes ✕ No	Hospice	Yes* ✕ No
Diabetes	Yes ✕ No	*Explain	
Insulin	Yes ✕ No	Car	Yes ✕ No
Diabetes related Oral Meds	Yes ✕ No	Service Animal	Yes ✕ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 30 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Harbor Crest Medical Center (fictional)
Department/Floor: 6 East Medicine
Social Worker/Case Manager/Nurse: Keith Yamamoto, LCSW
Phone: 555-0141
Email: kyamamoto@harborcrest.example
Nursing Station: 555-0142
Authorized By: Dr. P. Adeyemi

Patient Information

Name: Luis Doe
MRN: HC-5027719
Birth Date: 06/30/1972
Gender: M
Ethnicity: Hispanic or Latino
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: Spanish / English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: MSSA bacteremia, L foot cellulitis
Admit Date: 09/30/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes **[X]** No [ ]
PPD/TB Test or Chest X-Ray: Date: 09/30/2026 Result: CXR clear

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes **[X]** No [ ] MSSA bacteremia - IV cefazolin via PICC
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other:

Length of Stay Authorized

Days: 30

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

Referring Party Comments

Home infusion arranged for IV antibiotics via PICC.

-- FICTIONAL DEMO RECORD - not a real patient --

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS – AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) – Medi-Cal
Member
Member Name: Luis Doe
DOB: 06/30/1972
Member ID: OCH-48816352
Referring Provider
Facility: Harbor Crest Medical Center (fictional)
Clinical
Primary Diagnosis: Sepsis due to methicillin susceptible Staphylococcus aureus
ICD-10: A41.01
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 10/06/2026
Requested Days: 30
Plan Use Only
Status: Pending review

-- FICTIONAL DEMO RECORD – not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Luis Doe
DOB: 06/30/1972
Member ID: OCH-48816352

Referring Provider

Facility: Harbor Crest Medical Center (fictional)

Clinical

Primary Diagnosis: Sepsis due to methicillin susceptible Staphylococcus aureus
ICD-10: A41.01
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/06/2026
Requested Days: 30

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --



Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Harbor Crest Medical Center (fictional) - PATIENT FACE SHEET

Patient: Luis Doe
MRN: HC-5027719
Account #: 99027719
DOB: 06/30/1972
Sex: Male
Preferred Language: Spanish / English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/30/2026 14:45
Admit Type: Inpatient
Unit/Bed: 6E-604
Attending: Priya Adeyemi, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL

Date: 09/30/2026 16:30

CC

L foot redness and swelling x 5 days, fever and chills

HPI

54 y/o M, unhoused (sleeping at a bus shelter), with blisters on L foot from ill-fitting shoes that became red, swollen and painful over 5 days. Fevers and rigors since last night. Denies IVDU.

ED: T 102.9F, HR 118, BP 108/64, WBC 19.1. L dorsal foot erythema to ankle, no open wound, no fluctuance.

XR L foot: soft tissue swelling, no bone erosion, no gas.

PMH

- Hypertension (not on meds for 1 year)

PSH

- None

Meds PTA

None

Allergies

NKDA

Social

Unhoused x 2 years. Tobacco 1 ppd. Denies EtOH or drug use. UDS negative.

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date: 09/30/2026 16:30

CC

L foot redness and swelling x 5 days, fever and chills

HPI

54 y/o M, unhoused (sleeping at a bus shelter), with blisters on L foot from ill-fitting shoes that became red, swollen and painful over 5 days. Fevers and rigors since last night. Denies IVDU.
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- Hypertension (not on meds for 1 year)

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Meds PTA

None

Allergies

NKDA

Social

Unhoused x 2 years. Tobacco 1 ppd. Denies EtOH or drug use. UDS negative.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam
Gen: ill-appearing, AOx4
CV: tachycardic, no murmur. Lungs: clear
L foot: dorsal erythema and warmth extending to ankle, tender, intact skin, healing blisters, no drainage
Assessment/Plan
L foot cellulitis with sepsis.
1. Blood cultures x2, then vancomycin + cefazolin IV
2. MRI L foot to evaluate for deep infection / osteomyelitis
3. MRSA nares screen
4. Dispo: unhoused, CM consult for recuperative care
Signed: Priya Adeyemi, MD 09/30/2026 17:20

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

Exam

Gen: ill-appearing, AOx4
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L foot cellulitis with sepsis.
1. Blood cultures x2, then vancomycin + cefazolin IV
2. MRI L foot to evaluate for deep infection / osteomyelitis
3. MRSA nares screen
4. Dispo: unhoused, CM consult for recuperative care
Signed: Priya Adeyemi, MD 09/30/2026 17:20

-- FICTIONAL DEMO RECORD - not a real patient --

Infectious disease consult

INFECTIOUS DISEASE CONSULT
Date: 10/02/2026 10:15
Reason: MSSA bacteremia
Blood cultures 09/30: MSSA 2 of 2 bottles. Repeat blood cultures 10/01 and 10/02: no growth to date.
MRI L foot 10/01: cellulitis of dorsal foot, no abscess, no osteomyelitis.
TTE 10/02: no vegetation, EF 60%.
Recommendations
- Stop vancomycin. Cefazolin 2 g IV every 8 hours.
- Complicated MSSA bacteremia (fever >72h before clearance): total 4 more weeks IV cefazolin via PICC, end date 11/03/2026.
- PICC placement once repeat cultures negative x 48h.
- Home infusion and home health nursing needed for PICC care and weekly labs (CBC, BMP).
- ID clinic follow up 2 weeks after discharge.
Signed: R. Marsh, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 7: Infectious disease consult

other | orientation normal | legibility good

INFECTIOUS DISEASE CONSULT

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- PICC placement once repeat cultures negative x 48h.
- Home infusion and home health nursing needed for PICC care and weekly labs (CBC, BMP).
- ID clinic follow up 2 weeks after discharge.

Signed: R. Marsh, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#6
Date: 10/05/2026 08:00
S: Foot pain 2/10. No fevers. Eating well.
O: T 98.5F HR 78 BP 128/80 RR 16 SpO2 98% RA. AOx4.
L foot: erythema resolved to small area over dorsum, no warmth.
A/P: MSSA bacteremia from L foot cellulitis, cleared.
- Afebrile since 10/03
- Cefazolin 2 g IV every 8 hours via R arm PICC (placed 10/04) through 11/03/2026
- PICC tip confirmed SVC on CXR 10/04
- Amlodipine 5 mg PO daily started 10/03 for HTN, BP controlled
- Dispo: medically ready once home infusion / home health confirmed
Signed: Priya Adeyemi, MD

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 8: Hospitalist progress note

progress_note | orientation normal | legibility good

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- PICC tip confirmed SVC on CXR 10/04
- Amlodipine 5 mg PO daily started 10/03 for HTN, BP controlled
- Dispo: medically ready once home infusion / home health confirmed

Signed: Priya Adeyemi, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing note

NURSING NOTE
Date: 10/05/2026 06:50
Pt AOx4, cooperative.
Pt up in room and to bathroom independently, no device.
Showered independently with PICC cover. Toileting independently, continent.
R arm PICC: dressing clean, dry, intact; flushes without difficulty.
Pt self-administered amlodipine 5 mg PO at 06:00 with RN observation; states purpose and schedule.
Signed: S. Delgado, RN

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 9: Nursing note

nursing_note | orientation normal | legibility good

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Signed: S. Delgado, RN

-- FICTIONAL DEMO RECORD - not a real patient --



Case management note

CASE MANAGEMENT NOTE
Date: 10/05/2026 10:30
Pt unhoused, agreeable to recuperative care.
Home infusion referral sent 10/03 to Seaside Infusion Services (fictional) 555-0144 for cefazolin and PICC care.
Home health nursing referral sent 10/03 to Valley Bloom Home Health (fictional) 555-0188 for weekly PICC dressing change and labs.
Status 10/05: neither agency has accepted yet. Awaiting insurance authorization review.
Self-administration teaching cannot start until infusion agency accepts.
ID clinic follow up to be scheduled after discharge date set.
Signed: Keith Yamamoto, LCSW

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 10: Case management note

social_work | orientation normal | legibility good

CASE MANAGEMENT NOTE

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Self-administration teaching cannot start until infusion agency accepts.
ID clinic follow up to be scheduled after discharge date set.
Signed: Keith Yamamoto, LCSW

-- FICTIONAL DEMO RECORD - not a real patient --

Vitals, labs and microbiology

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/03 20:00 | 98.9 | 84 | 130/82 | 16 | 98% | RA
10/04 08:00 | 98.4 | 80 | 126/78 | 16 | 98% | RA
10/04 16:00 | 98.7 | 82 | 132/80 | 16 | 97% | RA
10/04 23:00 | 98.2 | 76 | 124/76 | 16 | 98% | RA
10/05 08:00 | 98.5 | 78 | 128/80 | 16 | 98% | RA

LABS

09/30 | WBC 19.1 H | Hgb 13.4 | Plt 266 | Na 135 | K 3.9 | Cr 1.1 | Glucose 112
10/04 | WBC 9.0 | Cr 0.9

MICROBIOLOGY

09/30 Blood cultures x2: Staphylococcus aureus, methicillin susceptible (MSSA), 2 of 2
10/01 Blood cultures x2: no growth to date
10/02 Blood cultures x2: no growth to date
09/30 MRSA nares PCR: Negative
SARS-CoV-2 swab: to be collected on day of discharge (not yet done)

IMAGING

09/30 CXR: no acute process
10/01 MRI L foot: cellulitis, no abscess, no osteomyelitis
10/04 CXR: R PICC tip in SVC

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 11: Vitals, labs and microbiology

labs | orientation normal | legibility good

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/03 20:00 | 98.9 | 84 | 130/82 | 16 | 98% | RA
10/04 08:00 | 98.4 | 80 | 126/78 | 16 | 98% | RA
10/04 16:00 | 98.7 | 82 | 132/80 | 16 | 97% | RA
10/04 23:00 | 98.2 | 76 | 124/76 | 16 | 98% | RA
10/05 08:00 | 98.5 | 78 | 128/80 | 16 | 98% | RA

LABS

09/30 | WBC 19.1 H | Hgb 13.4 | Plt 266 | Na 135 | K 3.9 | Cr 1.1 | Glucose 112
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MICROBIOLOGY

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10/01 Blood cultures x2: no growth to date
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IMAGING

09/30 CXR: no acute process
10/01 MRI L foot: cellulitis, no abscess, no osteomyelitis
10/04 CXR: R PICC tip in SVC

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list (draft)

DISCHARGE MEDICATION LIST - DRAFT

Patient: Luis Doe MRN: HC-5027719

- Cefazolin 2 g IV every 8 hours via PICC through 11/03/2026 (home infusion - pending agency acceptance)
- Amlodipine 5 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain
- Nicotine patch 21 mg/24h daily

Lines

- R arm PICC (placed 10/04/2026) - remove at end of therapy

Follow up

- ID clinic 2 weeks after discharge (to be scheduled)

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 12: Discharge medication list (draft)

med_list | orientation normal | legibility good

DISCHARGE MEDICATION LIST - DRAFT

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- Nicotine patch 21 mg/24h daily

Lines

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Follow up

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-- FICTIONAL DEMO RECORD - not a real patient --